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Name: احلام محمود عبده عبدالرحمن National ID: 27604221400482	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

احلام
محمود عبده عبدالرحمن
العمار الكبرى
مركز طلوع - القليوبية

١٩٧٦/٠١/٢٢ ٢٧٦٠٤٢٢١٤٠٠٤٨٢

JU1367592

٢٠٢٣/٠١ ٢٧٦٠٤٢٢١٤٠٠٤٨٢

حاصله على دبلوم تجاره

أنثى مسلمة متزوجة
اشرف عبدالجليل احمد الشاذلي

البطاقة سارية حتى ٢٠٣٠/٠١/٢٨

١١

Patient's Name: Ahlam Mahmoud Abdo
Date: 2026-04-20
Referred by: Prof.Dr. CAP
Hospital: Goustave Roussey International Egypt

- Spiral Multi-slice Non contrast C.T Examination of the Chest Abdomen and Pelvis Revealed:
- Compared to the study dated 18-9-25 current study revelas

CHEST

- Stationary course as regrad the right breast distortion and skin thickening reaching 1mm
- Still noted subcentmetric axillary lymph nodes largest 9mm
- Regression of the previously noted pulmonary nodules with multiple right lung upper and lower lobe thin fibrotic bands , Clear left lung field with no evidence of focal pulmonary infiltration or masses.
- No evidence of pleural or peri-cardiac sac collection.
- No mediastinal or hilar lymph nodes seen
- Normal CT appearance of the heart and its great vessels.

ABDOMEN AND PELVIS

- Normal size of both liver showing stationary course as regard the right lobe hepatic small hypodense focal lesion 3-5 mm in segment VIII.
- No IHBRDs
- Both kidneys are of normal site and size showing bilateral renal cortical cystic changes
- Normal CT appearance of the pancreas, aorta , IVC & spleen with splenic focal capsular calcification likely old granuloma .
- The uterus is of average size with homogenous CT texture. No focal lesions.
- No sizable adnexa or ovarian masses.
- No gross pelvic masses or collection.
- No significant pelvic lymphadenopathy.
- Normal CT study of the urinary bladder.
- Still seen left 5th to 8th rib Heald fractures

CONCLUSION:

Stationary to regressive course of the previously noted chest and abdominal finding for cliical correlation and further assessment .

DR ABUBAKR HEJAZI MSc FEBR

Name:	احلام محمود عبده	
MR. :	2269	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The multidisciplinary tumor board on 22 December 2025 discussed maintenance therapy with intravenous herceptin and a rechallenge with tamoxifen for a patient with metastatic breast cancer (c7 herceptin IV, tamoxifen, zoledronic acid) pending laboratory results. For cycle 10 the patient received herceptin 440 mg IV plus tamoxifen 20 mg; an echocardiogram was scheduled for the same day. The patient's oncologic history since 2023 includes a T1 N0 M0, locally advanced right-sided breast cancer treated with neoadjuvant chemotherapy TCHP for six cycles (completed 2 December 2023), followed by radiation therapy and adjuvant herceptin for one year and tamoxifen planned for five years. A bone scan on 4 March 2025 demonstrated multiple bilateral active metastatic rib lesions, while CT of chest/abdomen/pelvis showed only bone disease. The patient subsequently received six cycles of taxol and five cycles of intravenous herceptin, achieving stable disease on CT and a bone scan reported as free of disease. She is now maintained on intravenous herceptin and was rechallenged with tamoxifen for cycle 11 of herceptin, although tamoxifen approval for a new CT CAP was pending. A CT CAP was pending; tamoxifen was unavailable, and there was no approval for intravenous herceptin on that day, with zoledronic acid administered only. Laboratory studies were within normal limits. Tamoxifen was prescribed with supportive therapy for one month of herceptin IV; the order remained pending. Tamoxifen and supportive care were continued for two months, with the next clinic visit scheduled for 6 August 2026. The patient's disease is estrogen receptor negative (ER 0/8), progesterone receptor 3/8, HER2 + 3, with metastatic bone disease after full management including a 17-week taxol regimen that was stopped at the patient's request. The note emphasizes that there is no role for tamoxifen. At cycle 11 the patient received intravenous herceptin, tamoxifen, and supportive care. At cycle 12 the patient continued intravenous herceptin, tamoxifen, and supportive care, and subsequently proceeded with cycles 13 and 14 in the current phase. The patient has now completed cycles 12, 13, and 14 of herceptin IV with tamoxifen and supportive therapy.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
نهى سعيد علي محمد		الطبيب المعالج :
يعتمد ,		

name	W. S. S. (A/P)	MRN	11111
		ID	512
	47	Phone	11111
menopausal status	Post Men	Marital status	
SCORE		Family history	
		REASON OF MDT	20
comorbidity			

Clinical Data

				R
6/8	PR: 3/4	HER 2: Hx3+	KI67:	
mammography:				 TNM
is Workup: NCAJ. TCHP At Wt + ALMO → Ad Hcp lot 31 24y wson TAM				

Recommendations:

bas - 31 24y → bt. ach Met lo

for total week / hercept

OUTCOME & PLAN

① For total weekly + Hercept

② Zometa / 3 months

Note (ask by patient)

SIGNATURE

CLINICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
4/4	Dr. A. S. S.				
	8/7/2025				

Patient Name: Ahlam Mahmoud Abdou Abd El Rahman	Date : 18/09/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 49 Year(s)
Medical ID: 2381/2023	Pat. ID: 44771

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED: -**

**known case of right CBS. Compared to previous study at 9-4-2025.*

I. Chest:

- Stationary course regarding :
 - Right breast edema, distorted architecture and skin thickening (1.2 cm).
 - Multiple prominent right axillary lymph nodes the largest about 0.9x0.8 cm.
 - Right upper lobe (anterior segment) and middle lobar pleural thickening, reticulations and traction bronchiectatic changes, likely post radiotherapy sequel.
 - Few bilateral mostly pleural based / subpleural tiny pulmonary nodules, the largest reaches 4mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II. Abdomen and pelvis:

- The liver is seen enlarged showing no appreciable time interval changes regarding the number and size of the previously noted two tiny rather cystic hypodense focal lesions at segment VIII ranging from 3-5 mm.
- Patent portal vein as well as its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Tiny gall bladder dense stone.
- Normal size & parenchymal texture of the spleen with peripheral calcified focal lesion likely sequel of old granulomatous infection.
- No pathological abdominal or pelvic lymph nodes.
- Stationary course regarding the bilateral multiple renal cystic lesions, otherwise average parenchymal thickness & density with no stones or back-pressure changes.
- Normal plain CT appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted left 5th and 7th ribs healed fractures.

Dr. Sherif Wagdy

Patient Name	: الأستاذة / احلام محمود عبده عبدالرحمن	Request No.	: 13004968
Patient No.	: 27604221400482	Request Date	: 28/12/2023
Path No.	: 4590/23	Verification Date	: 11/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data : Right breast cancer post NACT

Nature of the specimen : Right WLE& Right ALN biopsy .

Gross Examination:

Three specimens were received :

1- Rt WLE specimen measured 8 x7x2.5 cm , covering skin ellipse measured 5.5x2 cm .
Cut section revealed : wired area :

- well defined greyish white firm tumour bed site measured 2.5 x1.5x1 cm , near to medial margin (away by 3 cm, 1 cm, away from least (lateral, superior, inferior) margins, 0.3 cm from deep margin.

3 - ALN biopsy : A fibrofatty tissue measured 11x5x2 cm enclosing 24 ALNs

• Frozen section-Free margins

Microscopic examination:

• Histological type: No viable tumor

• Angiolymphatic invasion: No

• Lymph node involvement: Free ALNs (0/24)

• Resection Margins: Free margins

• Histologic response to therapy: Pathologic complete response

• Additional pathologic findings: Microcalcification

Diagnosis:

Right wide local excision & Right ALN biopsy :

No viable tumor in a previously diagnosed invasive duct carcinoma

• Free margins

• Free ALNs (0/24)

• Pathologic complete response

Verified by :

Dr. Maha Hasar.

Printing Date : 13/01/2024 11:59:52

Page 1 of 2

Printing By : .marwa mohamed youss

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name :	الأستاذة / احلام محمود عبده عبدالرحمن	Request No. :	13004968
Patient No. :	27604221400482	Request Date :	28/12/2023
Path No. :	4590/23	Verification Date :	11/01/2024
Sex :	Female	Financial Category :	اورام دار السلام - نفقة الدولة
Age :	47 Y	Department :	Out Patient
Referring Doc :		Registered by :	Ali mostafa

Pathology Report

Stage : ypT0 , ypN0

Signature

Prof. Dr. Gamal Abdelatty
Dr. Heba Hassan

Verified by :

Dr. Heba Hasan

Printing Date : 13/01/2024 11:59:52

Page 2 of 2

Printing By : marwa mohamed youss

Patient Name	: الأستاذة / احلام محمود عبده عبدالرحمن	Request No.	: 13006363
Patient No.	: 27604221400482	Request Date	: 26/06/2023
Ref No.	: SR-746/23	Verification Date	: 11/07/2023
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

clinical Data: Right breast cancer.
History of Specimen: Slide revision

Request Examination:

Reparaffin block labeled by (5484/22) and three immunostained slidea were received.

Microscopic Examination:

Histological type: Invasive duct carcinoma

Grade (differentiation+ nuclear pleomorphism + mitosis): Grade II (3+2+1).

Immunohistochemistry: No nuclear reaction in tumor cells (Allred score 0/8).

Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).

2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Right breast mass, Slide revision:

Invasive duct carcinoma, Grade II.

Negative (score 0/8).

Positive (score 3/8).

2: Positive (score 3+).

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by :

Dr. Hebatallah Mostafa

Printing Date : 27/12/2023 10:07:21

Page 1 of 2

Printed By : .marwa mohamed youss

Patient Name : الأستاذ / احلام محمود عبده عبدالرحمن	Request No. : 13006363
Patient No. : 27604221400482	Request Date : 26/06/2023
Id No. : SR-746/23	Verification Date : 11/07/2023
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 47 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer.
Reason of Specimen: Slide revision

History Examination:

A paraffin block labeled by (5484/22) and three immunostained slidea were received.

Microscopic Examination:

Histological type: Invasive duct carcinoma

Grade (differentiation+ nuclear pleomorphism + mitosis): Grade II (3+2+1).

1: No nuclear reaction in tumor cells (Allred score 0/8).

2: Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).

3: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Invasive duct carcinoma, Slide revision:

Invasive duct carcinoma, Grade II.

Negative (score 0/8).

Positive (score 3/8).

2: Positive (score 3+).

Signature

Dr. Rana Fathy

Dr. Menan Hossam

Verified by :

Dr.Hebatallah Mostafa

Printing Date : 27/12/2023 10:07:21

Page 1 of 2

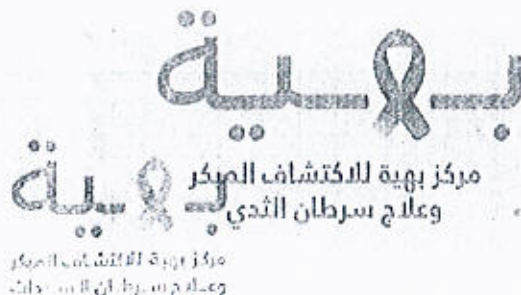
Printed By : .marwa mohamed youss



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قسم الأشعة التشخيصية Baheya I M S Certified



PATIENT NAME: AHLAM MOHMOUD ABDO ABD ELRAHMAN	PATIENT ID #: 20223000579
GENDER: Female	ORDER #: 2337539
AGE: 47 y	DATE OF SERVICE: 22-Jun-2023 11:32 AM

Report:

99mTc-MDP BONE SCAN

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 minutes & 2 hours after I.V administration of 20 mCi 99mTc-MDP :

Findings:-

Degenerative changes at dorso- lumbar vertebrae for follow up.

Arthritic changes at knees and shoulders.

Otherwise, no hot or cold spots that would account for serious bone lesions.

Both kidneys are seen.

CONCLUSION: -

No definite evidence of metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed

22-Jun-2023 05:19 PM

PATIENT: AHLAM MOHMOUD ABDO ABD ELRAHMAN, 20223000579, Female, 47 y

Report version 1, 25-Jun-2023 10:01 AM

Page 1 of 1





Patient Name: Ahlam Mahmoud Abdou Abd El Rahman	Date : 09/04/2025
Exam : C.T CHEST WITH CONTRAST	Age : 49 Year(s)
Medical ID: 2381/2023	Pat. ID: 44771

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

**known case of right CBS. Compared to previous study at 21/9/2024.*

I. Chest:

- Stationary course regarding :
 - Right breast edema, distorted architecture and skin thickening (1.2 cm).
 - Multiple prominent right axillary lymph nodes at level I, the largest about 0.9x0.8 cm.
 - Right upper lobe (anterior segment) and middle lobar pleural thickening, reticulations and traction bronchiectatic changes, likely post radiotherapy sequel.
 - Few bilateral mostly pleural based / subpleural tiny pulmonary nodules, the largest reaches 4mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II. Abdomen and pelvis:

- The liver is seen enlarged showing Stationary course regarding number and size of previously noted at least two tiny rather cystic hypodense focal lesions at segment VIII ranging from 3-5 mm.
- Patent homogeneous portal vein as well as its main tributaries
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Tiny gall bladder dense stone.
- Normal size & parenchymal texture of the spleen with peripheral calcified focal lesion likely sequel of old granulomatous infection.
- No pathological abdominal or pelvic lymph nodes .



Patient Name: Ahlam Mahmoud Abdou Abd El Rahman	Date : 09/04/2025
Exam : C.T CHEST WITH CONTRAST	Age : 49 Year(s)
Medical ID: 2381/2023	Pat. ID: 44771

- Bilateral renal multiple cystic lesions are seen , the largest one measuring (4.8x5.2 cm) with thin septations BOKNIAK II for follow up , otherwise average parenchymal thickness & density with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Newly seen left 7th rib lateral segment partially united fracture ? post Traumatic. Otherwise, No worrisome lytic or sclerotic bony lesions.

OPINION:

- **Rather stationary course since last study**

Dr.Ahmed Elgazar

Dr. Walid Mohamed, M.D.

Name : Ahlam Mahmoud Abdu Abdulrahman

Date : 22-06-2026 08:52

Branch : Dokki

Medical No. : 399571

Ref.Physician Prof.: Goustav Roussey Egypt

Whole Body Tc-99m MDP Bone Scintigraphy Report

Clinical History

A 50-year-old female patient with a history of right breast carcinoma, status post-mastectomy, currently presents with localized skeletal pain.

Technique: Whole-body planar skeletal scintigraphy (Anterior & Posterior views)

Findings

- **Axial Skeleton:** No focal areas of pathologically increased or decreased radiopharmaceutical uptake could be identified.
- **Appendicular Skeleton:** Both shoulder girdles, the pelvic ring, and the long bones of the upper and lower extremities show regular, symmetric tracer accumulation without evidence of focal osteoblastic activity.
- **Soft Tissue & Excretion:** Normal, symmetric prompt renal excretion is noted bilaterally, with expected physiological activity within the urinary bladder. The background soft tissue clearance is within normal limits.

Impression

- **Considering the prior study dated 22-6-2023:** Still no abnormal active or cold focal bony lesions that could account for metastatic bone disease.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine